

Care Plan Review
Prepared for your next provider meeting.

PLAN OVERVIEW

Provider	Capital Aged Care Services Pty Ltd
Effective	01/01/2027 → 31/03/2027
Classification	5
Quarterly budget	\$9,924.25

PLAN SUMMARY

This is a Support at Home care plan for Robert, Classification 5. It covers 01/01/2027 to 31/03/2027. The plan provides 2 services across Clinical, Independence, with a quarterly budget of \$9924.25. Overall, the plan appears to meet the flagship checks. See the findings below.

VERIFICATION CHECKS

Five Support at Home checks we run on every plan. A pass is confirmed correct, not just silence.

Care management fee · CONFIRMED

Care management fee correctly calculated at 10% of the care services base (\$900.00 on \$9000.00). AT-HM correctly excluded from the base.

AT-HM ring-fence · CONFIRMED

AT-HM allocation of \$1200.00 correctly ring-fenced from the quarterly budget and the care management base.

Personal care placement · CONFIRMED

Personal care correctly placed in Clinical Care post 01/10/2026 reclassification, with 0% participant contribution.

Rollover cap · CONFIRMED

Rollover cap correctly set at \$1000.00.

Quarterly budget vs classification · CONFIRMED

Quarterly budget of \$9924.25 matches the INDEX-1 Classification 5 value (\$9924.25) for the effective quarter.

FINDINGS SUMMARY

0 Compliance	0 Choice	0 Efficiency	9 Info
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VERBATIM QUESTION SCRIPT

Read each question aloud during the meeting. Space is provided below each for the provider's answer.

1. Robert has an assessed need for weekly laundry help and it appears in his goals, but I cannot see domestic assistance in the services delivered section. Can you explain why laundry is not included in the plan?

2. The goals look like they reflect what Robert wants. Can you confirm these were his exact words, and will he have a chance to update them at each review?

3. Robert lives alone and has clinical care needs. What happens if he needs help outside normal business hours, and is there an after-hours contact number or emergency plan?

4. Robert wants to get back to the men's shed and lawn bowls. Does the social support group session include transport to these activities, or do we need to add a separate transport service to the plan?

5. Can you confirm the care management fee calculation is based on \$9,000 of care services, and that the \$1,200 AT-HM allocation is excluded from that calculation?

6. The services listed add up to about \$1,656 per quarter, but Robert has a \$9,924 quarterly budget. Can you explain how the rest of the budget will be used to meet his assessed needs?

7. Robert has 30 minutes of nursing per week for wound care. Is this enough time for a full dressing change and assessment each visit, and what happens if the wound needs more frequent monitoring?

8. Robert's daughter was involved in the consultation. Does she provide any informal care for Robert, and if so, has the provider considered support for her as a carer?

9. The plan says it will be reviewed every 12 months. Given Robert has an active leg ulcer and clinical care needs, would a shorter review period like six months be more appropriate?

ALL FINDINGS

Grouped by severity, each finding cites the specific instrument. Take this list to the meeting alongside the questions above.

INFO · 9 findings

Domestic Assistance Not Included Despite Assessed Need · Service mix · confidence: low

Robert has an assessed need for weekly laundry assistance because he cannot manage heavy loads. The plan lists laundry in the assessed needs section and Robert states clean clothes and bedding as a personal goal. However, no domestic assistance service is included in the services delivered section. The Aged Care Rules 2025 require services to match assessed needs and participant goals.

Participant Goals Recorded in First Person · Participant voice · confidence: low

Robert has four goals written in his own words starting with 'I want'. This meets the participant voice requirement and shows the plan was developed with his input. The goals cover safety, clinical outcomes, daily living, and social participation.

No After-Hours Coverage Listed for Participant Living Alone · Cultural safety and cohort · confidence: low

Robert lives alone at age 78 with a clinical care need and an active wound. The plan does not mention after-hours contact arrangements or emergency support. For participants living alone with clinical needs, after-hours arrangements are an important safety consideration and should be documented.

Transport Not Included for Off-Site Social Participation · Service mix · confidence: low

Robert wants to return to the men's shed and his Tuesday lawn bowls group. Both activities are likely off-site. The plan includes one session per week of social support group but does not list any transport service. Without transport, Robert may not be able to reach these community activities, making the social support goal difficult to achieve.

Care Management Fee Appears Within Cap · Budget · confidence: low

The care management fee is \$900 per quarter, which is 10% of the \$9,000 care services base. This appears to comply with the 10% cap. The AT-HM allocation of \$1,200 is correctly ring-fenced and not included in the care management fee calculation.

Services Total \$1,656 Against \$9,924 Quarterly Budget · Budget · confidence: low

The three services listed total \$1,656 per quarter, which is 17% of the \$9,924 quarterly budget. The care management fee is \$900, bringing total utilisation to \$2,556 or 26%. This leaves \$7,368 unallocated each quarter. For a classification 5 participant with multiple assessed needs including laundry and social participation, this utilisation appears low.

Nursing Hours May Be Insufficient for Wound Management · Clinical adequacy · confidence: low

Robert receives 0.5 hours per week of nursing for wound care following a leg ulcer. Chronic wound management often requires assessment, dressing changes, and monitoring that may need more than 30 minutes per week, particularly if the wound is active or at risk of recurrence. The plan does not specify wound status or reassessment triggers for nursing hours.

Daughter Involved in Consultation · Cultural safety and cohort · confidence: low

Robert's daughter was present at the plan consultation on 15/12/2026. The plan does not describe her role, whether she provides informal care, or whether she has support needs of her own. If she is an informal carer, the plan should acknowledge this and consider her wellbeing.

Review Cadence Set to 12 Months · Time-bound triggers · confidence: low

The plan states it will be reviewed every 12 months or on changed needs. Robert has a clinical care need (wound management) and lives alone. A 12-month review interval may be too long for a participant with an active clinical condition. More frequent reviews (for example, quarterly or six-monthly) may be appropriate.

YOUR MEETING NOTES

Jot the provider's answers, any follow-ups, and decisions taken.

[illegible]

The Statement of Rights, National Aged Care Quality Standards, and Aged Care Rules 2025 provide the legal framework for this review. This artefact is a conversation prompt, not a formal audit.

Take this to your next provider meeting. Wayly Care Plan Review is a preparation aid, not a formal audit.

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